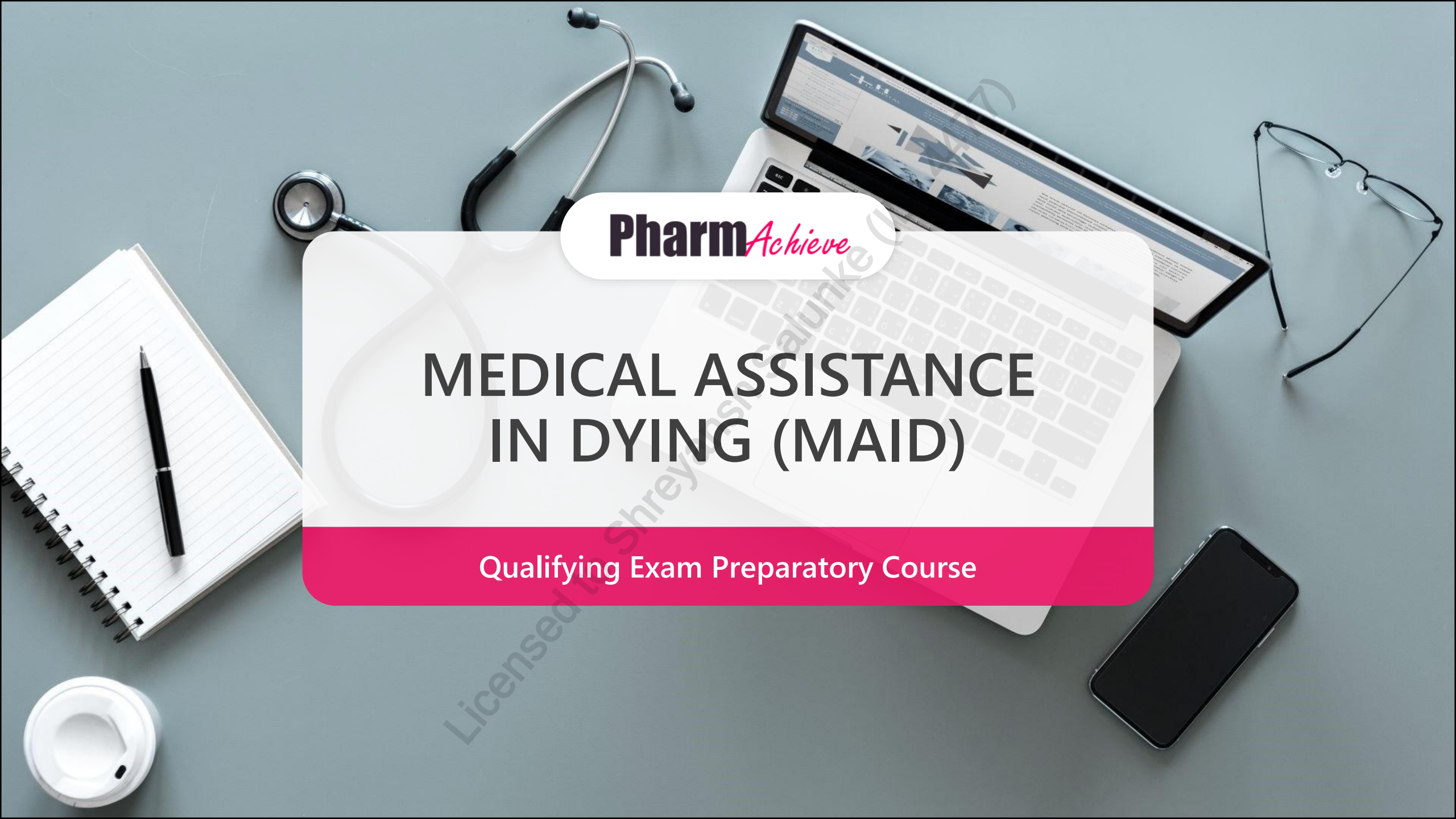




Pharm*Achieve*

MEDICAL ASSISTANCE IN DYING (MAID)

Qualifying Exam Preparatory Course

COMPETENCIES COVERED IN THIS PRESENTATION...

**Providing Care:
Clinical Care**

**Providing Care:
Distribution**

**Knowledge
& Expertise**

**Communication
& Collaboration**

**Leadership &
Stewardship**

Professionalism

WHAT IS MEDICAL ASSISTANCE IN DYING (MAID)

- As of June 2016, eligible Canadians are legally entitled to participate in MAID
- This allows for patients who are suffering from a **physical illness** to seek medical help to end their life
- **Physicians, nurse practitioners or the patient themselves** (except in Quebec) can administer the medications
- The patient may choose to have a memorial service prior to MAID occurring, and can choose to listen to music/prayers, etc. during the process

ELIGIBILITY CRITERIA

Patients must meet the following criteria to request MAID:

- Age ≥ 18 years AND have decision-making capacity (mental competence)
- Eligible for public health care
- Request it themselves **without external influence**
- Provide informed consent (at the time of request *and* immediately before receiving MAID)
- **Have a disease that is serious and incurable or a disability (does *not* have to be a terminal illness)**
- Have irreversibly declining capabilities
- Suffer from long-lasting, intolerable physical or psychological concerns that can't be improved in a way the patient finds acceptable

The request must be made in writing and signed and dated by the patient and an independent witness. The request can be withdrawn anytime in any manner.

Temporary exclusion criteria: If an individual's *sole* health condition is mental illness, they are not eligible to receive MAID until March 17, 2027

PROCEDURE

- Two independent health care professionals (physicians or nurse practitioners) must evaluate the patient to determine if they qualify for MAID.
 - 'Independent' means they do not know each other or the patient, and there is no potential for personal benefit
- Patient will be asked for final consent again right before the medication is administered

Additional procedures if the patient's natural death is **not** expected to occur in the reasonably foreseeable future:

- At least one of the professionals must have expertise in the illness that the patient is suffering from
- The patient must be informed about other methods to help with their suffering, like counselling, community support services, etc.
- Eligibility assessment is approximately 3 months

MEDICATIONS USED IN MAID

A kit is created which usually contains*:

- Midazolam-anxiolytic
- Lidocaine-anesthetic
- Propofol-anesthetic and coma-inducing agent
- Rocuronium-neuromuscular blockers to stop respiration

A backup kit may be needed in case of failure

** Medications used may differ depending on whether the patient or physician/nurse practitioner administers the medications*

PHARMACIST'S ROLE

- Pharmacists may be asked to dispense a prescription for the purposes of MAID to be used in the hospital, long-term care facility, etc.
- Pharmacists may be approached by patients or long-term care residents for more information on MAID
- Pharmacists may be asked to help create policies and procedures for their workplace on MAID
- Pharmacists may be asked to speak to patients' families about their decision and explain what will happen and how (the doctor will usually advise and support the patient and their family)
- It is important to ensure that the pharmacist does not impose their personal/religious views on the patient. If the pharmacist does not agree with the patient undergoing MAID, they can refer them to a health care provider who is willing and able to assist them

TAKEAWAY

Canadians are legally entitled to participate in MAID if they are suffering from a physical illness and seek medical help to end their life

Patients must meet the eligibility criteria to request MAID

The request is made in writing, signed and dated by the patient and an independent witness

Two independent healthcare professionals (physicians or nurse practitioners) evaluate whether a patient qualifies for MAID, with one being an expert in the disease state

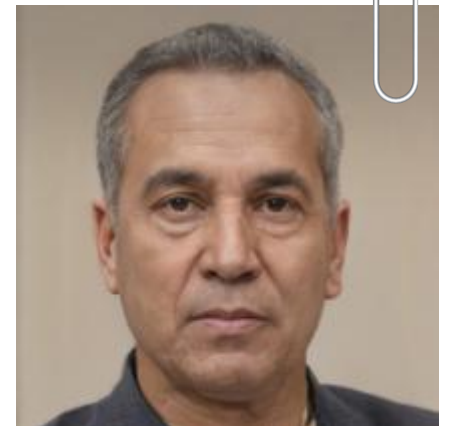
Medications used in MAID may include: **midazolam** (anxiolytic), **lidocaine** (anesthetic), **propofol** (anesthetic and coma-inducing agent) and **rocuronium** (neuromuscular blocker)

Pharmacists may be asked to dispense a prescription for the purposes of MAID

CASE STUDY

TP is a 74-year-old male who has end-stage amyotrophic lateral sclerosis (ALS). He has been experiencing progressive muscle weakness, loss of mobility, odynophagia, and increasing respiratory compromise. After lengthy discussions with his family and healthcare team, TP has opted for medical assistance in dying (MAID). Two physicians independently assessed his case and after 4 months, TP was deemed to be eligible to receive MAID. TP provided informed, written consent, alongside a witness. He chose clinician-assisted MAID to take place in a hospital setting. A memorial service will take place in the hospital prior to the administration of the MAID kit. TP's family will be in attendance, and they will deliver speeches and a prayer during the service. TP's final consent will be obtained immediately prior to the medications being administered. Your community pharmacy has received a prescription from the medical team responsible for administering MAID to TP. You are expected to compound, dispense, and provide appropriate documentation according to federal and provincial regulatory guidance.

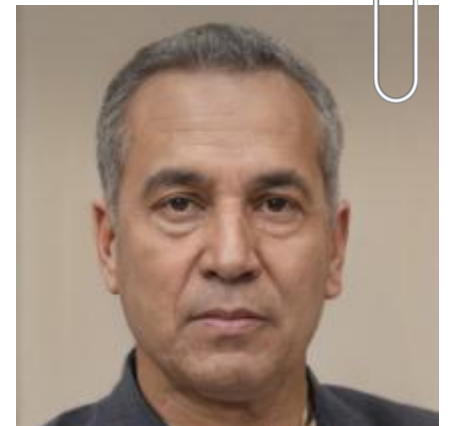
1. Which of the following statements accurately describes the pharmacist's legal role in the provision of MAID?
 - a) Pharmacists may administer MAID drugs if trained
 - b) Pharmacists are responsible for witnessing the patient's consent
 - c) Pharmacists may compound and dispense MAID drugs but may not administer
 - d) Pharmacists must obtain a second opinion about patient eligibility when a MAID prescription is received



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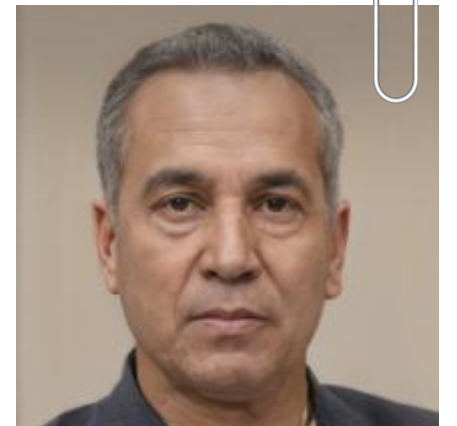
2. Which of the following is true regarding the ethical obligation of pharmacists if they object to participating in MAID?
- a) Refuse to fill the prescription and destroy it
 - b) Notify the patient and refer them to another pharmacy or provider
 - c) Ignore the request until another staff member handles it
 - d) Dispense the medication but record your objection in writing



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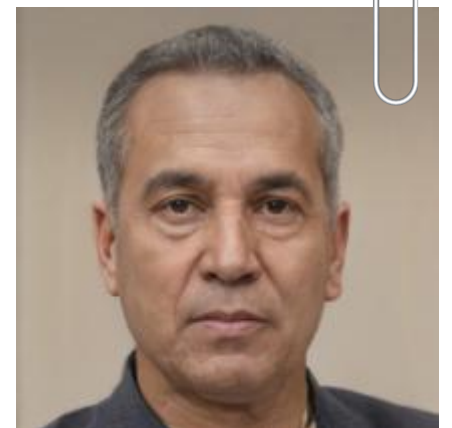
3. Which of the following medications are typically contained within a MAID kit?
- a) Propofol, rocuronium, midazolam, lidocaine
 - b) Benzocaine, hydromorphone, diazepam, levobupivacaine
 - c) Fentanyl, lorazepam, diphenhydramine, glycopyrrolate
 - d) Ketorolac, digoxin, naloxone, chlordiazepoxide



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4. Which of the following actions is most appropriate if a pharmacist receives an incomplete or unclear prescription intended for MAID?
- a) Return the prescription and ask the prescriber to send a new one
 - b) Contact the prescriber directly to clarify and resolve the issue before dispensing
 - c) Fill the prescription assuming normal MAID protocols
 - d) Contact the patient to confirm the intended drugs and doses



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1. Canada's medical assistance in dying (MAID) law. Government of Canada. Last modified 2024-06-19. <https://www.justice.gc.ca/eng/cj-jp/ad-am/bk-di.html>
2. Stukalin I, Olaiya OR, Naik V, et.al. Medications and dosages used in medical assistance in dying: a cross-sectional study. CMAJ Open. January 2024. [https://www.cmajopen.ca/content/10/1/E19#:~:text=The%20Canadian%20Association%20of%20MAiD,neuromuscular%20blockers%20to%20stop%20respirati on\).](https://www.cmajopen.ca/content/10/1/E19#:~:text=The%20Canadian%20Association%20of%20MAiD,neuromuscular%20blockers%20to%20stop%20respirati on).)
3. Get the facts: Canada's medical assistance in dying (MAID) law. Dying with dignity Canada. 2021. <https://www.dyingwithdignity.ca/end-of-life-support/get-the-facts-on-maid/>.
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CHANGE LOG

July 2025

- Formatting changes throughout
- Slide 8: Takeaway slide added
- Slides 9, 10: Case study and questions added

October 2025

- Minor formatting edits, no content changes made

March 2026

- Slide 2: Updated NAPRA competencies
- Slides 4, 5: Minor updates